

D-SNP INTEGRATION: FIDE, HIDE AND COORDINATION-ONLY

Use the integration level to locate responsibility - never to promise a benefit | Verified through July 24, 2026

Integration level answers: How are Medicare and Medicaid contracts, benefits, enrollment and member processes connected? It does **not** replace the member's plan documents.

LEVEL	FEDERAL STRUCTURE	WHAT IT MEANS ON A CALL	VERIFY BEFORE ANSWERING
FIDE SNP Fully Integrated Dual Eligible SNP	One entity holds the MA and Medicaid MCO contracts. The Medicaid contract covers broad categories: primary/acute care and Medicare cost sharing, LTSS, behavioral health, home health, and medical supplies/equipment, to the extent available under Medicaid. Uses aligned care management and integrated member processes. For 2025+, requires exclusively aligned enrollment and whole-service-area Medicaid contracting.	Most integrated federal D-SNP category. The same organization may manage both sides, but the member's actual coverage, network, authorizations and carve-outs still control.	Plan ID and state; affiliated Medicaid MCO; aligned enrollment; benefit category; current handbook/EOC; any CMS-approved carve-out.
HIDE SNP Highly Integrated Dual Eligible SNP	The MA organization or a related entity has a capitated Medicaid contract covering LTSS or behavioral health . For 2025+, that contract must cover the D-SNP's entire service area. CMS may approve limited carve-outs.	Substantial integration, but not simply "FIDE-lite." Some Medicaid categories may remain outside the D-SNP arrangement. HIDE status alone does not prove exclusively aligned enrollment.	Which category is integrated: LTSS, behavioral health or both; Medicaid MCO; AIP/EAE status; excluded services; plan-specific routing.
COORDINATION-ONLY D-SNP	A D-SNP that is not FIDE or HIDE. It still has a State Medicaid Agency Contract and coordinates Medicare/Medicaid, including required information-sharing for designated high-risk members.	Coordination does not mean the D-SNP pays every Medicaid claim. Medicaid benefits may remain with the state or another Medicaid plan/entity.	Member's Medicaid carrier/program; responsible entity for the service; crossover/cost-sharing rules; referral or transfer pathway.
ALIGNED ENROLLMENT	The member is enrolled in a D-SNP and an affiliated Medicaid managed care plan under the same parent/arrangement.	AIP = Applicable Integrated Plan	A D-SNP/Medicaid arrangement with exclusively aligned enrollment that uses integrated appeals and grievance processes. AIP is not interchangeable with FIDE or HIDE.
EXCLUSIVELY ALIGNED	All D-SNP enrollees receive Medicaid benefits through the affiliated Medicaid MCO arrangement.	2026 SEP REMINDER	The Integrated Care SEP may be used monthly only to create aligned enrollment into a qualifying FIDE, HIDE or AIP D-SNP and its affiliated Medicaid MCO.

SIX-STEP CALL CHECK

1. Plan ID + state 2. Integration level 3. Medicaid MCO/alignment 4. Benefit owner: Medicare, Medicaid or both 5. Current plan source 6. Explain and document

SAFE MEMBER LANGUAGE: "Your plan coordinates Medicare and Medicaid. Which services Aetna administers depends on your exact plan and state. I am checking the current plan information before I quote the benefit."

NEVER SAY: "Because you have a D-SNP, Aetna covers all Medicaid services" or "FIDE/HIDE means every service has no cost."

Primary authorities: 42 CFR 422.2, 422.107 and 422.561; CMS D-SNP Integration page and CY 2026 enrollment guidance; Aetna 2026 FIDE/HIDE materials. **Status:** Peer-created quick reference; not Aetna-approved; plan documents and controlled internal procedures govern.